
AUA 2026

Advanced & Metastatic Prostate Cancer

Narrative update with abstract digests and original poster appendix

Washington, DC | May 2026

Speaker line

The useful update is not a list of new agents. It is a cleaner way to run an advanced prostate cancer clinic.

72

advanced-session items

IP38 + PD23 + IP73

6

original posters

attached as source pack

40

guideline recs

AUA/SUO 2026 amendment

Not one breakthrough. A new operating model.

AUA 2026 connects treatment intensification, imaging, genomics, and toxicity management into one clinical workflow.

INTENSIFY

Doublet therapy is no longer exotic; the real question is who needs triplet therapy.

SELECT

The triplet signal is clinically plausible, but not uniform enough to become an automatic reflex.

IMAGE

PSMA PET is central, yet post-RLT monitoring may need more than one tracer story.

SUSTAIN

The endpoint is not only response. It is whether the clinic can keep patients safely on therapy.

Talk track

I would present this as a clinic-design update: under-treatment and over-treatment now happen in the same room, and the answer is workflow discipline.

The guideline update validates the story.

The May 2026 AUA/SUO amendment explicitly expands guidance across imaging, molecular testing, risk stratification, and treatment paradigms.

IMAGING

Appropriate imaging becomes a treatment-selection tool, not just a staging formality.

TESTING

Germline and somatic testing move earlier and become part of routine APC counseling.

RISK

Risk stratification is needed to choose intensity, de-intensity, and sequencing.

TREATMENT

Combination therapy and sequencing are now a guideline-level management problem.

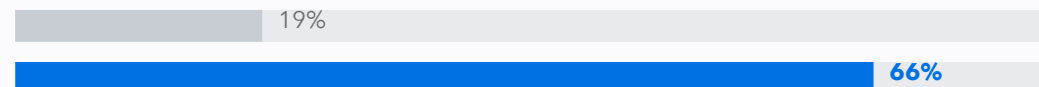
Why this matters

The 040IC course description mirrors the same pattern: genetic counseling, somatic testing, ADT + ARPI + chemotherapy, PSMA PET, PARP inhibitors, CRPC sequencing, and de-intensification.

Routine care has already moved away from ADT alone.

In a Swedish population-based de novo mCSPC cohort, doublet and triplet uptake rose alongside improved 3-year survival.

Upfront doublet

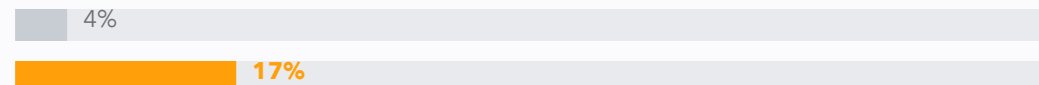


9,294

de novo mCSPC men

NPCR Sweden, 2016-2024

Upfront triplet



3-year OS, all men



3-year OS, age <65



INTERPRETATION

The signal is real-world adoption plus survival movement, not randomized proof that every patient needs triplet therapy.

Triplet therapy needs a gate, not a reflex.

Bayesian and real-world data keep ADT + ARPI as the rational default while identifying a narrower high-volume, fit-patient lane.

OVERALL

HR 1.04

Pr benefit 36%

HIGH-VOLUME

HR 0.89

Pr benefit 90%

MEANINGFUL BENEFIT

HR <0.8

19% in high-volume

Selection gate

Consider triplet when high-volume disease, good performance status, chemo-eligible physiology, and patient goals align.

Otherwise, intensify with ADT + ARPI and preserve later-line options.

YUSHIMA registry: triplet was not clearly superior for time to CRPC; Bayesian borrowing suggested modest RMST benefit that depended on external evidence.

The first ADT decision still varies by pathway.

Ontario data suggest intensification improved over time, but real-world delivery still depends on patient barriers and care-system design.

3,871

patients

de novo mHSPC, age ≥ 66

332

urologists

ADT initiated by urologist

26.5%

intensified

ARPI or docetaxel within 6 mo

Mean uptake per urologist

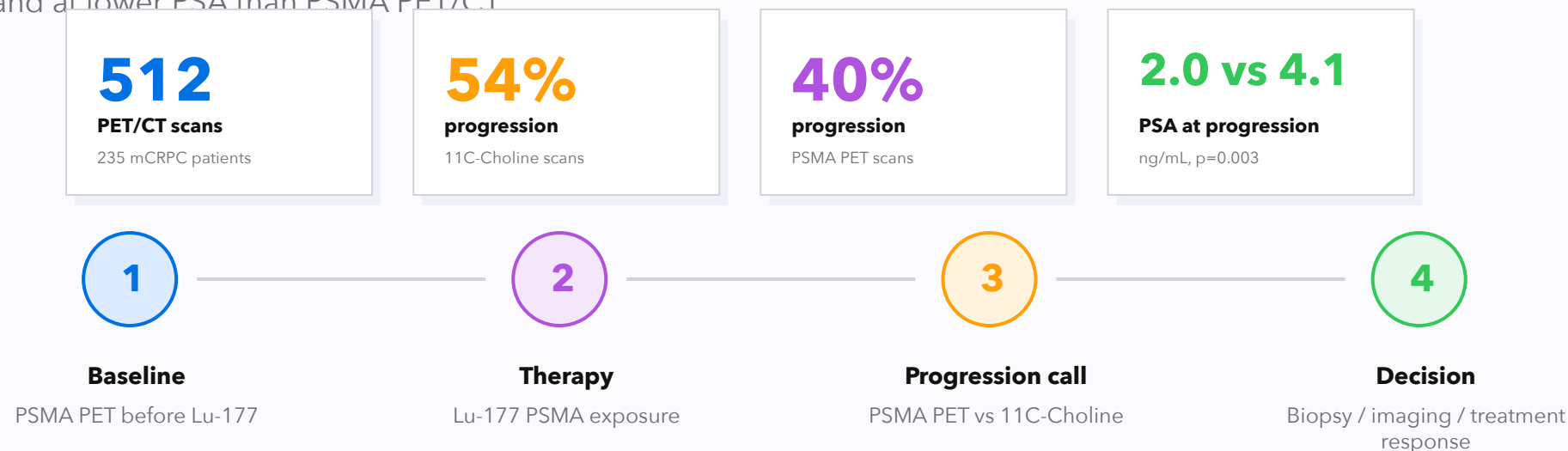


NARRATIVE POINT

The gap is no longer knowledge alone. It is referral timing, frailty/comorbidity assessment, drug access, documentation, and whether the urology clinic has a default intensification pathway.

After target pressure, PSMA is no longer the whole imaging story.

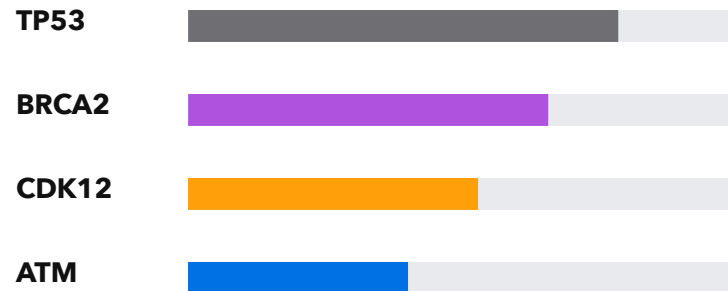
A Mayo registry analysis suggests 11C-Choline PET/CT may detect post-Lu-177 PSMA progression more often and at lower PSA than PSMA PET/CT



BRCA is no longer a binary checkbox.

A large Japanese CGP cohort argues for variant-level annotation and treatment interpretation beyond simply 'BRCA altered' or 'not altered'.

CGP landscape



Clinical readout

- BRCA1 PVs: inferior outcomes vs BRCA2 PVs
- BRCA2 loss: most favorable olaparib response signal
- MTB review shifted earlier in recent cases
- NEPC phenotype remains a separate escape problem

PRACTICE POINT

Order testing early, but read the result deeply: specimen, timing, pathogenicity class, zygosity/loss, CDK12/ATM context, and neuroendocrine phenotype all change the discussion.

The hidden endpoint is clinic durability.

AUA 2026 toxicity posters make the same point: intensified therapy fails if adverse effects are handled as afterthoughts.

270

PC360 enrolled

5-domain ADT care model

20.3%

MRONJ at 3 yr

denosumab cohort signal

Supportive-care operating model

Cardiometabolic

Bone / dental

Mood / cognition

Anemia / QoL

Sexual health / exercise

INTERPRETATION

The endpoint is not only response; it is whether patients stay functional, monitored, and able to complete the intended treatment sequence.

Talk track

The modern APC clinic needs standing workflows for bone, metabolic risk, mood, cognition, anemia, sexual health, exercise, dental review, and treatment completion.

What I would change after AUA 2026.

A practical APC workflow for the Monday clinic: make the default visible, document the exceptions, and audit the pathway.

01	mHSPC default	Discuss ADT + ARPI early for eligible de novo metastatic patients.
02	Triplet gate	Document volume, symptoms, fitness, chemo eligibility, and patient preference.
03	PSMA/RLT rule	Predefine how progression will be called after Lu-177 PSMA exposure.
04	Variant read	Capture germline, somatic, specimen, pathogenicity, and zygosity/loss.
05	Toxicity clinic	Build ADT/RLT supportive care into the pathway, not the rescue visit.

BOTTOM LINE

AUA 2026 is a prompt to redesign the advanced prostate cancer clinic around evidence selection, source documentation, and toxicity capacity.

IP38-12 is the backbone abstract for the narrative.

Use this when someone asks why the deck says intensification is now routine practice.

IP38-12

Uptake of doublet and triplet therapy for men with de novo metastatic castration-sensitive prostate cancer. Population-based study

Question	How much did upfront doublet/triplet treatment enter routine care, and did survival move in parallel?
Population	9,294 men with de novo mCSPC in Sweden's NPCR, 2016-2024.
Key result	Doublet uptake 19% -> 66%; triplet 4% -> 17%; 3-year OS 51% -> 61%; among age <65, 69% -> 77%.
Take-home	Real-world treatment intensity increased, supporting early intensification while leaving patient selection unresolved.

The supporting abstracts make the workflow argument.

These are the source-level pieces that turn a drug update into a clinic-model update.

IP38-13

Triplet selection signal

Overall OS advantage was unclear; high-volume patients had higher probability of benefit, but clinically meaningful benefit remained modest.

IP73-14

Implementation variation

Only 26.5% of older Ontario mHSPC patients received intensification; uptake rose to 59% by 2022, showing pathway improvement but not uniform delivery.

IP73-16

Post-Lu-177 PSMA imaging

¹¹C-Choline detected progression more frequently and at lower PSA than PSMA PET/CT in a retrospective post-RLT registry.

IP73-24

Variant-level CGP

Variant-level BRCA interpretation provided prognostic and predictive information beyond binary BRCA status.

IP38-17

PC360 ADT toxicity pathway

A multidisciplinary ADT care model enrolled 270 men and operationalized five domains of supportive care.

IP38-12: Uptake of doublet and triplet therapy in de novo mCSPC

Rendered from the original local poster PDF; included as source appendix for internal presentation use.

Uptake of doublet and triplet therapy for men with *de novo* metastatic castration sensitive prostate cancer. Population-based study

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AUA-2026
Washington, DC
MAY 15-18



INTRODUCTION

- In randomized clinical trials doublet and triplet therapy improved survival compared to standard androgen deprivation therapy (ADT) in men with *de novo* metastatic castration-sensitive prostate cancer (mCSPC).
- Guidelines recommend **doublet therapy since 2020** and **triplet therapy since 2022**.
- The aim of this study was to assess the uptake of upfront doublet and triplet therapy at a population level and assess trends in survival for all men with mCSPC.

MATERIALS AND METHODS

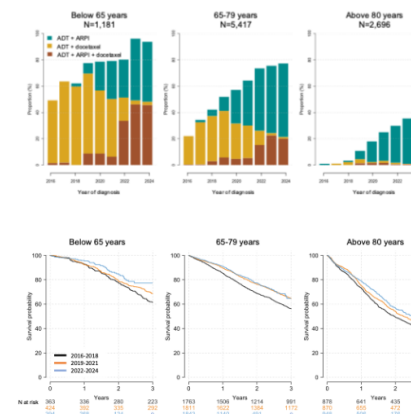
- We included men registered with **de novo mCSPC** in **2016-2024** in the National Prostate Cancer Register (NPCR) of Sweden.
- Use of **Docetaxel** within 6 months after diagnosis was extracted from both NPCR and regional health care IT systems. Use of androgen receptor pathway inhibitors (ARPIs) was based on filled prescriptions for abiraterone, enzalutamide, apalutamide and darolutamide in the Prescribed Drug Register.
- We estimated the annual proportion of men with *de novo* mCSPC who upfront received doublet therapy (ADT plus ARPI or docetaxel) or triplet therapy (ADT plus docetaxel and ARPI).
- Kaplan-Meier curves were used to estimate 3-year **overall survival** in three calendar periods (2016-2018, 2019-2021, and 2022-2024).

CONCLUSION

The uptake of doublet and triplet therapy increased substantially during the study period, in particular among men below age 65. In parallel, 3-year overall survival increased in all men diagnosed with *de novo* mCSPC. These data provide support for benefit of upfront doublet or triplet therapy also in routine clinical practice.

RESULTS

- Median PSA at diagnosis decreased slightly from 104 ng/mL (IQR: 28-434) in 2016-2018 to 89 ng/mL (IQR: 25-354) in 2022-2024.
- In **9,294** men diagnosed with *de novo* mCSPC the use of upfront **doublet therapy** increased from **19% in 2016** to **66% in 2024**, and use of **triplet therapy** rose from **4% in 2021** to **17% in 2024**.
- Men treated with triplet therapy were younger and had a more advanced disease, reflected by **high PSA levels, clinical T stage and Gleason score**.
- Three-year survival increased from **51% (95% CI: 49-52%)** in 2016-2018 to **61% (95% CI: 58-64%)** in 2022-2024. Among men below age 65, survival increased from 69% (95% CI: 65-73) in 2019-2021 to 77% (95% CI: 71-84) in 2022-2024.



IP38-13: Bayesian reanalysis of triplet vs doublet therapy in mHSPC

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AUA 2026
Washington, DC

Abstract #IP38: Does Triplet Therapy Improve Survival in Metastatic Hormone-Sensitive Prostate Cancer? A Bayesian Individual Patient Data Reanalysis

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Institute of
SCIENCE TOKYO

INTRODUCTION

- Triplet therapy (ADT + docetaxel + ARAT) vs doublet (ADT + ARAT) remains controversial in mHSPC.
- Prior rIPD analyses showed no significant OS benefit, but could not quantify probability of benefit.
- Objective:**
To quantify the probability that triplet therapy improves OS using a Bayesian reanalysis of phase III trial data.

METHODS

- Data:**
 - 8 phase III trials (ARASENS, PEACE-1, ARANOTE, TITAN, ENZAMET, ARCHES, STAMPEDE, LATITUDE)
- Models:**
 - Bayesian Cox proportional hazards model
 - Hierarchical Poisson GLMM
 - Noninformative prior: Normal(0,10)
 - Sensitivity analysis: weakly skeptical prior Normal(0,0.5)
 - Estimation via MCMC
- Key endpoints: OS**
 - Pr(HR < 1.0) → any benefit
 - Pr(HR < 0.8) → clinically meaningful benefit
- Additional:**
 - Tumor volume stratification (high vs low)
 - Treatment class comparison (AR inhibitor-based vs abiraterone-based triplets)
 - Secondary endpoints: rPFS and TTmCRPC

RESULTS

Overall Population

HR: 1.04 (95% CrI 0.92–1.18)

Pr(HR < 1): 36%

Pr(HR < 0.8): 11%

* HR, hazard ratio; CrI, credible interval

High-volume

HR: 0.89 (95% CrI 0.77–1.03)

Pr(HR < 1): 90%

Pr(HR < 0.8): 19%

* HR, hazard ratio; CrI, credible interval

Low-volume

HR: 0.91 (95% CrI 0.66–1.25)

Pr(HR < 1): 73%

Pr(HR < 0.8): 22%

* HR, hazard ratio; CrI, credible interval

Figure 1. Posterior distributions of overall survival comparing triplet vs doublet therapy. Points represent posterior medians; whiskers indicate 95% CrI. HR < 1 favors triplet therapy.

Group	Statistical Method	HR (95% CrI)	Pr(HR < 1.0)	Pr(HR < 0.8)
Low volume	One-stage Cox (Normal approx.)	1.04 (0.92–1.18)	72.6%	21.8%
	Poisson GLMM (Hierarchical)	1.04 (0.92–1.18)	71.9%	38.9%
High volume	One-stage Cox (Normal approx.)	0.89 (0.77–1.03)	94.6%	8.3%
	Poisson GLMM (Hierarchical)	0.89 (0.77–1.03)	95.0%	18.9%
Overall	One-stage Cox (Normal approx.)	1.04 (0.92–1.18)	25.7%	0.01%
	Poisson GLMM (Hierarchical)	1.04 (0.92–1.18)	26.0%	11.4%

Triplet therapy shows low probability of meaningful OS benefit overall, with possible modest benefit in high-volume disease.

CONCLUSIONS

ADT + ARAT doublet remains the rational default; triplet therapy may be considered selectively in high-volume patients.

- No clear OS advantage in the overall population
- Modest probability of benefit in high-volume disease
- Probability of clinically meaningful benefit remains low

DISCLOSURES

The authors have nothing to disclose.

Source: scraper/data/posters/IP38-13.pdf

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IP73-16: PSMA PET/CT and 11C-Choline after Lu-177 PSMA therapy

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MAYO CLINIC

PSMA PET/CT and 11C-Choline PET/CT for Evaluating Disease Progression Following Lutetium-177-PSMA Therapy in Metastatic Castration-Resistant Prostate Cancer

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IP73

Introduction and Background	Results		C-11 Choline SCAN (106)	PSMA PET SCAN (128)	P value
Prostate cancer is the most common cancer affecting men worldwide. Lutetium-177-PSMA (177Lu-PSMA) demonstrated a key role in management of advanced prostate cancer, especially in patients with castration-resistant disease who have failed hormonal and chemotherapy treatments. In light of studies reporting low PSMA expression and tumor heterogeneity in patients undergoing treatment with Lutetium which raises concerns on using only PSMA PET/CT in monitoring patient's response. Herein, we sought to assess post Lutetium-177-PSMA radiographic progression using PSMA PET and Choline PET scans.	Table 1 summarizes clinicopathological variables. Median age, Gleason score, prior treatments, and baseline disease distribution were similar between both groups. Among 195 ¹¹C-Choline PET/CT scans, 106 (54%) demonstrated radiographic progression, compared with 128 (40%) of 317 PSMA PET/CT scans. Median PSA at radiographic progression was 2.0 ng/mL for ¹¹C-Choline PET/CT and 4.1 ng/mL for PSMA PET/CT (p = 0.003).	Age at Diagnosis of PCa,	71.09	71.64	0.3135
		median (IQR), years	(66.643-76.052)	(66.836-77.340)	
		Primary Gleason Score, median (IQR)	7	7	0.56
		(8 - 9)	(8 - 9)		
		Median of PSA (IQR) at DX	11.365	12.3	0.81
		(5.6-37.8)	(5.675-50)		
		Primary Treatment of PCa, n (%)			
		RP	55 (51.89%)	67(52.34%)	0.945
		RT	17 (16%)	18.75 (21.65%)	0.566
		Chemo hormonal	28 (26.42 %)	26 (20.31%)	0.271
		Unknown	7 (6.6%)	11(8.594 %)	
		Distribution of Mets disease before Lutetium			
		local	34 (32.08%)	31 (24.22%)	0.182
		Lymph Node	67 (63.21%)	76 (59.38%)	0.550
		Bone	83 (78.30%)	109 (85.16%)	0.1738
		viscera	17 (16.04%)	28 (21.88%)	0.259
		Median (IQR) of Lutetium cycles	6(6-6)	6 (5-6)	0.0003
		Median (IQR) of PSA at radiographic disease progression After Lutetium therapy	2.1	4	0.0034
		(0.155-10.3)	(0.87-29.195)		



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IP73-24: CGP landscape and BRCA1/2 pathogenic variants in mCRPC

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#IP73-24

AUA-2026
Washington, DC

Comprehensive Genomic Profiling Landscape and the Clinical Impact of BRCA 1/2 Pathogenic Variants in the Metastatic Castration-Resistant Prostate Cancer

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2) Division of Pharmacotherapeutics, Keio University Faculty of Pharmacy, Tokyo, Japan



Background

- ✓ Metastatic castration-resistant prostate cancer (mCRPC) shows marked clinical and genomic heterogeneity.
- ✓ Alterations in homologous recombination repair (HRR) genes, particularly *BRCA1/2*, are clinically actionable.
- ✓ PARP inhibitor olaparib has demonstrated survival benefit in *BRCA*-altered mCRPC (PROfound trial).
- ✓ Real-world data evaluating genomic heterogeneity and variant-specific outcomes remain limited, especially in Asian populations.
- ✓ This study aimed to characterize the genomic landscape of HRR alterations and evaluate the prognostic and therapeutic impact of *BRCA1/2* pathogenic variants in a nationwide Japanese cohort.

Methods

- ✓ Retrospective cohort study using the **C-CAT national precision oncology database** in Japan.
- ✓ Included **5,893 patients with mCRPC** who underwent comprehensive genomic profiling (CGP).
- ✓ Five reimbursed CGP platforms were analyzed (tumor tissue and liquid biopsy).
- ✓ HRR alterations defined based on pathogenic or likely pathogenic annotations (**12 HRR genes including *BRCA1*, *BRCA2*, *ATM*, *CDK12*, *CHEK2*, *PALB2*, *RAD51C*, *RAD51D*, *BRIP1*, *BARD1*, *FANCA*, and *FANCL***).
- ✓ **Primary endpoints:** Overall survival after olaparib initiation (OS_olaparib)
- ✓ Survival analyses performed using Kaplan-Meier and multivariable Cox regression models.

Results/Graphs/Data

Figure 1. Patient characteristics in total cohort

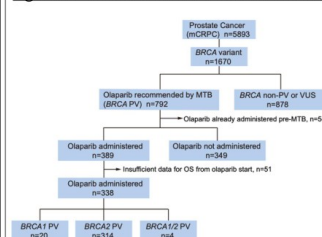


Figure 2. MTB timing and CGP specimen

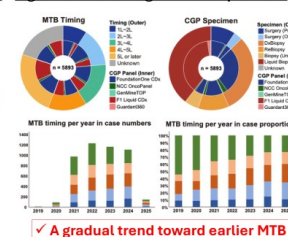


Figure 3. Genomic landscape in mCRPC

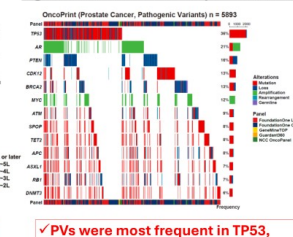


Figure 4. HRR gene alterations in prostate cancer

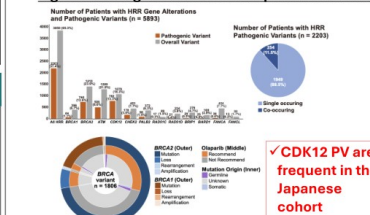


Figure 5. Survival analysis of prostate cancer patients with *BRCA1* or 2 PVs

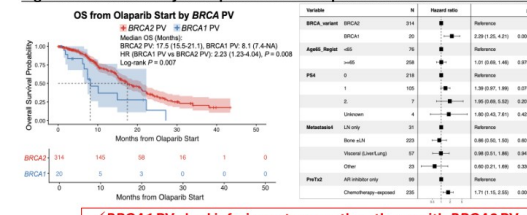
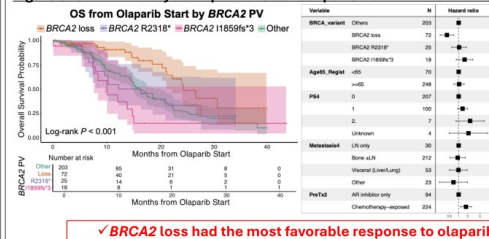


Figure 6. Survival analysis of prostate cancer patients with *BRCA2* PVs



Conclusion

Variant-level annotation of *BRCA* alterations offers clinically meaningful prognostic and predictive insights beyond *BRCA* mutation status alone in patients with mCRPC receiving olaparib.